Proposed foot action categories for Hajj pilgrims

For pilgrims with and without diabetes. Apply the highest applicable category. These are proposed action categories requiring clinical review and prospective validation; they are not established injury probabilities. [1-5]

Category	Findings after questions + foot examination	Action required
LOW CONCERN	Skin intact; no concerning symptoms; protective sensation and circulation assessed as normal; no significant prior foot disease; suitable footwear; reliable readings.	Foot-care advice, suitable footwear, avoid barefoot walking, daily self-inspection. [1-3]
MODERATE CONCERN	No high-risk findings, but footwear rubbing, unusually heavy walking, substantial callus/deformity, minor localized irritation or uncomplicated blister, or a repeatable unexplained sensor finding.	Reduce friction and loading; inspect and protect skin; adjust footwear; arrange reassessment if findings persist or worsen. [1-3]
HIGH CONCERN Priority clinical review	Loss of protective sensation; known or suspected poor circulation; previous ulcer, amputation or Charcot disease; or current open wound, worsening lesion, or lesion on a vulnerable foot.	Prompt clinician assessment. Same-day review for an active wound or worsening lesion; individualized offloading and walking plan. [1-3]
URGENT OVERRIDE	Spreading infection or systemic illness; new hot, red, swollen neuropathic foot; sudden cold/pale foot; significant trauma or inability to bear weight.	Stop routine device testing; arrange urgent or emergency medical assessment. Suspected Charcot requires clinician-directed immobilization/offloading. [4-5]
INCOMPLETE ASSESSMENT	Essential sensation/circulation examination missing, or temperature/pressure data unreliable.	Complete or repeat assessment; do not report “low concern.” Device safety rule; validate.

Measurement rules

A wound or urgent clinical finding takes precedence over normal sensor readings. The IWGDF >2.2°C left-right difference on two consecutive days applies to serial monitoring in selected people with diabetes; the <200 kPa target concerns validated in-shoe footwear offloading. Neither is a universal single-visit cutoff for all pilgrims. [2]

Clickable references

[1] APMA: Diabetic wound care

[2] IWGDF: Prevention guideline (2023)

[3] Foot injuries among Hajj pilgrims with and without diabetes (2014)

[4] IWGDF/IDSA: Diabetes-related foot infection guideline (2023)

[5] IWGDF: Charcot guideline (2023)